**جوستاف روسي انترناشيونال - مصر (EPE) *** **طلب علاج أول مرة / متردد**

**إسم المريض رباعياً :** سميحه لوقا خليل عبد الملاك

**المحافظة :** القاهره **الحي / المدينة :** المرج **العنوان :** 15م الصفا ش محمود لبنه

**رقم البطاقة :** 25609112600425 **رقم التليفون :** 01277755697 **تاريخ تسجيل الإستمارة :** 20-10-2026

**العمر(تاريخ الميلاد) :** 11-09-1956 **المهنة :** ربه منزل

---

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي المقر بما فيه**

**التوقيع:(----------)** **التوقيع:(----------)**

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)

**نتائج الأبحاث :** (مرفق)

**التشخيص النهائي :** سرطان بالثدي (مبادرة)

**خطة العلاج :** علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام

**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------

**خطة العلاج :** ----------

**المستشفي :** ----------**قسم :** ----------

**جهة العلاج :** ----------**نفقات :** ----------

**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

**Name:** سميحه لوقا خليل عبد الملاك
**National ID:** 25609112600425

## تقرير طبي

**السيد**

**تحية طيبة وبعد,,**

| | |
|---|---|
| **إيماء للطلب المقدم من** | **بشأن تقرير طبي للمريض** |
| **بقسم:** | **رقم طبي:** |

**نتشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | |
|---|---|
| **تاريخ وساعة الدخول :** -- --:-- yyyy-mm-dd | **تاريخ وساعة الخروج :** PM 01:17 05-07-2026 |

**وبعد الكشف والفحوصات والمتابعة وجد انة**

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

| | |
|---|---|
| **التاريخ :** **05-07-2026** | **الوقت :** **-- --:--** |
| | **الطبيب المعالج :** د/شيماء لاشين |







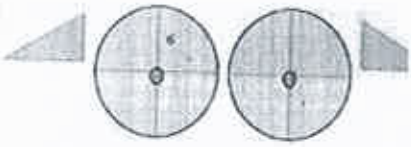
National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

# MDT form

A-Patient ID:
1- Name : سميحه لوقا خليل عبدالملاك
Sex: Female
3- Medical number: 83234067
Old Med :
Old Hos : 3297-2024
2-ID number:
Patient Age : 68 Y



| | | | | | | | |
|---|---|---|---|---|---|---|---|
| Family history | Family History | Phone | Phone 01277755697 | BMI | BMI | REASON OF MDT | REASON OF MDT POST OPERATIVE |
| marital status | Marital Status | NRS Score | NRS Score | GROUP | GROUP C | Date of MDT | Date of MDT 03/12/2024 |
| Co morbidity | Co Morbidity htn, hypothyroidi | Menopausal status | Menopausal Status POSTMENOPAUSAL | Initial STAGE | Initial STAGE | | |

## Clinical data

**Breast Imaging :**
It breast UIQ 1.5 x1 cm

**Mets Workup :**
cap: rt iliac bone & actebule of small density
PAUS:free

| BX | ER | PR | Her 2 Neu | SISH | Ki67 |
|---|---|---|---|---|---|
| IDC<br>BX COMMENT<br>gii with minimal DCIS | 8/8<br>ER COMMENT | 7/8<br>PR COMMENT | Her 2 Neu COMMENT<br>-ve | SISH COMMENT | 10 |

| T | N | M |
|---|---|---|
| 2 | 0 | 0 |

**Sonomammography**

**PATIENT HISTORY**
post CBS+SLNB upfront: IDC GII
free resection margins
LN 0/6

## MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
adjuvant 5 years AI
PORT
Zometa every 6 months

## SIGNATURE

| | | | |
|---|---|---|---|
| MEDICAL ONCOLOGY | MEDICAL ONCOLOGY | RADIOLOGY | RADIOLOGY |
| SURGERY | SURGERY | PATHOLOGY | PATHOLOGY |
| Psychiatric | Psychiatric | Nutrition | Nutrition |



MDT Form
[[DOCTYPE:MDT_FORM]]

| | | | |
|---|---|---|---|
| Patient Name | : الاسثلاء / سميحه لوفا خليل عبدالملاك | Request No. | : 13005535 |
| Patient No. | : 25609112600425 | Request Date | : 28/08/2024 |
| Path No. | : 3134/24 | Verification Date | : 02/09/2024 |
| Sex | : Female | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age | : 67 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : Mrs.Mayada Hegazy |

# Pathology Report

Clinical Data: Left breast suspicious mass.
Nature of Specimen: Tru-cut biopsy, collected at 27/08/2024.

Gross Examination:
- Four grayish white tissue cores up to 1.2 cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: Minimal.
* Perineural invasion: No.
* Angioinvasion: No.

Diagnosis:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with minimal DCIS.
For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :



غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / سميحه لوفا خليل عبدالملاك | Request No. | : 13005535 |
| Patient No. | : 25609112600425 | Request Date | : 28/08/2024 |
| Path No. | : 3134/24 | Verification Date | : 14/09/2024 |
| Sex | : Female | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age | : 67 Y | Department | : Out Patient |
| Referreing Doc. | : | IHC No. | : |
| | | Registered by | : Mrs.Mayada Hegazy |

# IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass
Nature of Specimen: Tru-cut biopsy

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with minimal DCIS.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :



غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

| | | | |
|---|---|---|---|
| Patient Name | الأستاذه / سميحه لوفا خليل عبدالملاك | Request No. | 13005535 |
| Patient No. | 25609112600425 | Request Date | 28/08/2024 |
| Path No. | 3134/24 | Verification Date | 14/09/2024 |
| Sex | Female | Financial Category | اوراء دار السلام - نفقة الدولة |
| Age | ٥٧ Y | Department | Out Patient |
| Referreing Doc. | | | |
| | | Registered by | Mrs.Mayada Hegazy |

# IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, ntermediate; and 3, strong). Total score from 0 to 8.

' Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of he invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr.Rana Fathy
Dr.Nermin Mohamed

Verified by :



غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

| | |
|---|---|
| Patient Name: Samiha Loka Khalil Abd Malak | Date : 26/09/2024 |
| Exam : ultrasound ABDOMEN& PELVIS | Age : 68 Year(s) |
| Medical ID: | Pat. ID: 79523 |

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:**

***Liver: -***

- *Enlarged in size (17cm at Rt MCL) bright parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

***Gall Bladder: -***

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

***Spleen: -***

*Average size, homogenous.*

- **Kidneys: -**
- *Both are average in size, shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *No gross lesion*

***Para-aortic and pancreatic area**: show no gross abnormalities.*

***No ascites***

***Urinary Bladder: -***

- *Not distended enough however no stones or diverticular out pouching*

***Uterus and adnexal**; no abnormalities could be detected*

*Opinion:*

***Bright hepatomegaly likely fatty***

Signature Dr . Nermien Ramadan

26/09/2024 2:21 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

| | | | |
|---|---|---|---|
| Patient Name | :الأستاذه / سميحه لوقا خليل عبدالملاك | Request No. | : 13002623 |
| Patient No. | :25609112600425 | Request Date | : 29/10/2024 |
| Path No. | :4161/24 | Verification Date | : 11/11/2024 |
| Sex | :Female | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age | :68 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical Data: Left breast cancer, Up-Front..
Nature Specimen: Left breast conservative surgery & SLNs.

Frozen Section:
- Negative lymph nodes (0/6).
- The margins were free grossly.

Gross Examination:
* Procedure: Left BCS & SLNs.
Fibrofatty tissue piece measured 8x7x2 cm with no covering skin.
* Tumor extent: A firm greyish white mass.
* Size: 2.2x2.1x1.5 cm.
* Distance from margins: 1 cm away from inferior margin, 1.5 cm from superior, lateral, superficial & deep margins, 2 cm from medial margin.
* SLNs: A fibrofatty tissue piece measured 4x3.5x1 cm, entangling 6 LNs up to 1x0.7 cm.
Designation:
1&2) Mass. 3) medial margin. 4) lateral margin. 5) superior margin. 6) inferior margin. 7) deep margin. 8) superficial margin. 9) SLNs.
Grossing by Dr. Toka Medhat.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (2+2+2).
* Ductal carcinoma in situ: Focal (papillary & solid patterns).
* Resection margins: Free.
* Perineural invasion: Not identified.
* Angioinvasion: Present.
* Microcalcifications: Not identified.

Verified by :
Dr.Rofayda Abdallah



غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / سميحه لوفا خليل عبدالملاك | Request No. | : 13002623 |
| Patient No. | : 25609112600425 | Request Date | : 29/10/2024 |
| Path No. | : 4161/24 | Verification Date | : 11/11/2024 |
| Sex | : Female | Financial Category | : نفقة الدولة - سلام |
| Age | : 68 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa moham |

## Pathology Report

- Lymph node involvement: Negative lymph nodes (0/6).
- Additional pathologic findings: Fibrocystic changes.

Diagnosis:
Left BCS & SLNs, Up-front:
Invasive duct carcinoma, Grade II.
- Free resection margins.
- Negative lymph nodes (0/6).
- Stage: pT2, pN0.

Signature
Prof. Amal Ahmed Hareedy.
Dr. Menna Nabil.
Dr. Rufaida Abdullah.

Verified by :
Dr.Rofayda Abdallah



غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Coraiche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــحة والسكـان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

| | |
|---|---|
| Patient Name: Samiha Loka Khalil Abd Malak | Date : 26/09/2024 |
| Exam : C.T ABDOMEN &PELVIS | Age : 68 Year(s) |
| Medical ID: 3297/2024 | Pat. ID: 79523 |

# NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

## Findings:

## I Chest:-

- Left breast parenchymal assemetry mounting to focal mass seen at IUQ measures 19x20 mm associated with fine parenchymal nodularity reaching retroareolar region
- Left multiple prominent level I and few level II axillary lymphnodes largest seen at the former measures 10x13mm
- Bilateral apical tiny pulmonary partially calcified nodules largest measures 5 mm
- No sizable areas of consolidation or cavitary lesions.
- Small subcentemetric prevascular and pretracheal lymphnodes, some showing internal hyersenisty likely calcific
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild global enlargment of the cardiac chambers. No pericardial effusion detected.
- Apart from ateromatous calcfiocations Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral small fibroelastoma dorsi are seen.

## II Abdomen & pelvis:

- The liver is seen mildly enlarged with diffuse homogenouslow parenchymal ct attenuation texture. Apart from tiny segemngt VI hyperdense likely calcified focal lesion about 5x5 mm likely old granulomatous sequele; No definte suspicious focal lesions (on non contrast base) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

| | |
|---|---|
| Patient Name: Samiha Loka Khalil Abd Malak | Date : 26/09/2024 |
| Exam : C.T ABDOMEN &PELVIS | Age : 68 Year(s) |
| Medical ID: 3297/2024 | Pat. ID: 79523 |

- Right gluteus maximus intramuscular fat containg leison measures 15x29mm likely lipoma

***Scanned skeleton:** right iliac bone / acdtabular roof small densely sclerotic lesion for bone scan.

**Opinion:**

- **Left breast malignant mass lesion**
- **Bilateral apical tiny nodules for follow up**
- **No evidence of hepatic metastatic deposits.**

**Dr/Reem Elwaly**

*Dr. Walid Mohamed, M.D.*

CT Scan
[[DOCTYPE:CT_SCAN]]